STARK MEMORIAL HOSPITAL

Advanced Trauma & Research Center | Malibu, CA

PATIENT DEMOGRAPHICS

Patient Name: Rogers, Steve
MRN: AGI-8992-X
DOB: 07/04/1920
Admission Date: 2025-11-21
Attending Phys: Dr. Anthony Stark, MD, FACC

CHIEF COMPLAINT

Patient presents with acute onset of fever, chills, and altered mental status. Family reports confusion starting 4 hours ago.

HISTORY OF PRESENT ILLNESS (HPI)

105 y/o male with history of super-soldier serum enhancement presents to ED. Vitals at triage: Temp 103.1F, HR 115, BP 90/55. Patient appears diaphoretic and lethargic. Initial concern for infectious etiology.

PHYSICAL EXAMINATION

GEN: Acute distress, tachypneic.
CV: Tachycardic, regular rhythm, no murmurs.
RESP: Clear to auscultation bilaterally, no wheezes.
ABD: Soft, non-tender.
EXT: Warm extremities, 2+ pulses.

LABORATORY RESULTS (COLLECTED 14:00)

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WBC:          18.5 H  (Ref: 4.5-11.0)
Hgb:          14.2    (Ref: 13.5-17.5)
Plt:          150     (Ref: 150-450)
Creatinine:   1.1     (Ref: 0.7-1.3)
Lactic Acid:  1.1     (Ref: 0.5-2.0)
Blood Cx:     Pending
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ASSESSMENT AND PLAN

1. SEVERE SEPSIS (R65.20)
   - Patient meets SIRS criteria (Tachycardia, Fever, Leukocytosis).
   - Started on IV Vancomycin/Zosyn.
   - 30mL/kg fluid bolus initiated.

2. Acute Encephalopathy
   - Likely secondary to infection.

DISCHARGE DISPOSITION: Admit to ICU.

Electronically Signed: Tony Stark, MD

Signed at 14:49:31